

OA

OUTPATIENT THERAPY SERVICES

Website Design & Content Brief

Phase 1 — Full Site Content, UX/UI Direction & WhatsApp Integration

| *Supporting Recovery. Promoting Independence. Enhancing Wellbeing.*

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For: Thami (build/implementation) — with sign-off from Theo Old-John, OA Outpatient Therapy Services

Reference materials used: OA Z-fold brochure (V6), OA Medical Centre Business Model & GTM document, OA referral partner presentation (v1 & v2)

Status: Phase 1 brief — content, IA, UX/UI direction, and WhatsApp integration. Phase 2 (booking systems, patient portal, WhatsApp automation) to follow separately.

1. How To Use This Brief

This document is the single source of truth for building the OA Outpatient Therapy Services website in Phase 1. It contains the finished, approved-direction content for every page, the site structure, and the UX/UI design direction needed to build a premium, conversion-focused site.

It is written so it can be handed straight to a developer or to Claude (via Claude Code, Claude for Chrome, or an artifact build) as a build prompt. Each page section below is broken into Section / Content so it can be lifted directly into components.

What's in scope for Phase 1

- A five-page marketing and information site: Home, About Us, Services (For Patients + For Practitioners as two dedicated pages), Resources, Contact Us.
- Finished, persuasive copy for every page — written to lead with the problem each audience faces, then resolve it with the OA offering.
- A site-wide WhatsApp entry point so patients, families, and referring professionals can reach OA in one tap.
- Full UX/UI direction: layout logic, brand system application, imagery direction, and interaction notes.

What's out of scope for Phase 1 (see Section 9 — Phase 2)

- Online booking / appointment scheduling system.
- Patient or referrer portal / login area.
- WhatsApp Business API automation (chatbot flows, auto-replies).
- E-commerce or online payment collection.

2. Website Objectives & Strategy

2.1 The core problem the site must solve

OA sits at a point of high anxiety and high stakes: a patient has just left hospital (or has been told they need structured therapy without being admitted) and has no clear next step. Separately, the doctors, hospital social workers, and allied health professionals who could refer to OA have no visibility of what happens to a patient once they leave their care, and no reason to trust a new outpatient provider with continuity of that care.

The website exists to remove both of those anxieties in under one minute of reading, for two very different audiences arriving with two very different questions:

Audience	Their question	What the site must prove
Patients & families	"I've just left hospital / I need help — now what do I do?"	OA is one coordinated programme, not five separate appointments to chase. It's covered by medical aid, and it comes to you.
Referring doctors & therapists	"If I refer this patient, do I lose them — or gain a partner?"	OA is an extension of the referrer's own care team, with a real feedback loop back to them.

2.2 Persuasive structure: problem, then solution

Every page that speaks to a decision (Home, both Services pages) follows the brochure's own proven structure — lead with the problem the visitor is living through, then present OA as the direct, specific answer to it. This is a deliberate departure from a generic “about our services” layout, and it should be protected through build and any future edits.

2.3 Conversion goals, in priority order

- 1. Start a WhatsApp conversation (the primary conversion action across the entire site, for both audiences).
- 2. Phone call to 081 358 2781.
- 3. Contact form submission (captures enquiries when WhatsApp/phone aren't convenient, and separates patient vs. professional enquiries).
- 4. Download of the practitioner one-pager / referral pathway summary (practitioner audience).

2.4 Brand voice

Warm, human, and direct — never clinical or corporate. Short sentences. Second person (“you”, “your recovery”). Confidence without hype: OA doesn't oversell, it removes uncertainty. This mirrors Theo's own direction on the brochure and referral deck.

3. Brand Foundations (Carried From Approved Collateral)

The website must feel like the same organisation as the Z-fold brochure and the referral partner presentation — not a new design pass. The tokens below are extracted directly from that approved collateral.

3.1 Colour palette

Colour	Hex	Role	Usage
Teal Dark	#0F3D40	Primary	Header/nav, footer, headline text, dark section backgrounds
Olive Green	#7A8F5E	Secondary / accent	CTAs, icons, dividers, stat numbers
Teal Light	#48959B	Accent	Eyebrow labels, links, secondary accents
Olive Tint	#E7EBDF	Background	Light section backgrounds, cards
Teal Tint	#E3EEEF	Background	Stat blocks, alternating sections
Off-white	#F7F7F4	Base	Page background

3.2 Typography

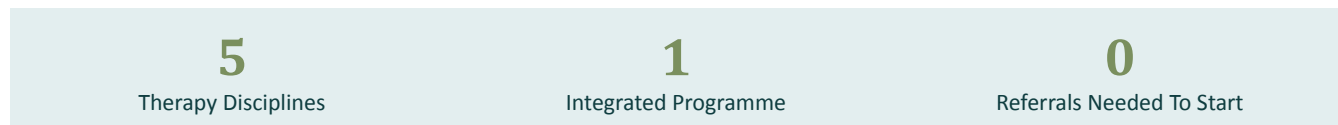
- Headings: Cambria (as used across the brochure and deck). Web-safe fallback/substitute if a licensed web font is preferred: a warm serif such as Merriweather or Lora — but confirm with Theo before substituting, since Cambria is now a recognised brand element.
- Body copy: Calibri, or a clean humanist sans web equivalent (Inter, Source Sans) at 16–18px minimum for body text — larger than typical, deliberately, given the recovery/older-adult audience (see 4.4 Accessibility).

3.3 Logo & tagline

- Interlocking-rings “OA” mark with “OUTPATIENT THERAPY SERVICES” lockup beneath, as used on the brochure and deck.
- Tagline, used sparingly as a closing/hero statement: “Supporting Recovery. Promoting Independence. Enhancing Wellbeing.”

3.4 Signature stat block

The brochure's 5 / 1 / 0 stat strip is a strong, ownable conversion device and should be reused prominently on the homepage:



3.5 Imagery direction

Per Theo's explicit direction (business model document): “pictures and video carry more power than words.” The site should lead with real photography — real patients (consented), real environments, real staff — never generic stock imagery of stretchers, sterile wards, or stock “doctor pointing at clipboard” shots. Group-session and transport imagery should be prioritised, as they are OA's strongest visual differentiators.

Discipline imagery is a hard requirement: each of the five discipline cards (Home and Services / For Patients) must show a real photograph of an OA practitioner actively working with a patient in that specific discipline — not an icon, not a stand-in stock photo. The five disciplines must also always be presented in the brochure's fixed order: Physiotherapy, Occupational Therapy, Clinical Psychology, Speech Therapy, Dietetics.

4. UX/UI Design Direction

4.1 Overall feel

Premium, calm, and human — closer to a boutique wellness/recovery brand than a medical directory. Generous whitespace, large full-bleed photography, and the brochure's confident split-panel layout logic (dark teal panel + photography + light content panel) carried into web sections.

4.2 Page structure logic

- Sticky header: logo left, primary nav centre/right, and a persistent, high-contrast “WhatsApp Us” button (olive or teal-light fill) — always visible, never buried in a hamburger menu on desktop.
- Hero sections built as split-screen (headline/CTA on one side, real photography on the other), echoing the brochure's title-panel treatment.

- Alternating section backgrounds (white / off-white / olive-tint / teal-dark) to pace long pages the way the Z-fold brochure paces its panels.
- Dual-path cards on the homepage (“For Patients & Families” / “For Referring Professionals”) as the primary navigation decision — this is the single most important UX moment on the site, since it routes two very different visitors correctly within seconds.

4.3 Interaction & motion

- Subtle scroll-reveal on section entry (fade/slide, ~300ms) — restrained, not gimmicky.
- Animated count-up on the 5 / 1 / 0 stat block when it enters view.
- Hover states on the five discipline cards (icon shift + light background tint) inviting exploration.
- The WhatsApp button should have a gentle pulse or subtle motion on first load only, not persistently — to draw the eye without becoming an irritant.

4.4 Accessibility & audience-specific usability

A meaningful share of visitors will be recovering from stroke, surgery, or illness, or will be older adults with reduced dexterity or vision — this must shape real decisions, not just a compliance checkbox:

- Minimum 16–18px body text; high colour contrast (teal-dark on off-white, not light-on-light).
- Large, thumb-friendly tap targets (minimum 44×44px) — especially the WhatsApp and Call buttons.
- Simple, shallow navigation — no more than one level of dropdown; the two Services pages should be visible as direct nav items, not hidden behind a hover menu.
- No auto-playing video with sound; no content that relies on colour alone to convey meaning.

4.5 Mobile-first requirement

Most patients and families will land on the site from a phone (often shared or borrowed while in hospital or a waiting room). Design and build mobile-first: a fixed bottom bar with “WhatsApp” and “Call” buttons should be present on mobile throughout the site, not just in the header.

5. Site Structure (Information Architecture)

Page	Purpose	Primary CTA
Home	Route two audiences fast; establish trust; introduce the 5 disciplines and the integrated model.	WhatsApp Us / choose your path
About Us	Tell the OA story and mission; build trust in the team and the coordinated-care model.	WhatsApp Us
Services → For Patients	Persuade patients/families: problem of fragmented recovery → OA's integrated solution.	WhatsApp / Call to start
Services → For Practitioners	Persuade referrers: problem of losing visibility post-referral → OA's MDT feedback loop.	Add OA to your referral pathway

Page	Purpose	Primary CTA
Resources	Educational content for both audiences; supports SEO and trust; softer, non-sales entry point.	Browse / WhatsApp a question
Contact Us	Convert any remaining visitor via form, phone, WhatsApp, or map/location.	Submit enquiry

Note: “Services” is a parent nav item that opens directly into the two dedicated pages (For Patients / For Practitioners) — visitors should never land on a generic, undifferentiated “Services” landing page, since the whole strategy depends on speaking to each audience directly.

6. Page-By-Page Content

6.1 Home

Hero	EYEBROW BENONI · JOHANNESBURG HEADLINE You've left hospital. Now what? SUBHEAD OA is the coordinated recovery programme that picks up exactly where hospital care ends — five therapy disciplines, one integrated programme, and zero referrals needed to begin. CTAS — Primary: “Chat to us on WhatsApp” (opens WhatsApp to 081 358 2781) — Secondary: “Call 081 358 2781”
Stat strip	Reuse the brochure's signature stat block directly beneath the hero: 5 Therapy Disciplines · 1 Integrated Programme · 0 Referrals Needed To Start
Dual-path decision	SECTION INTRO Whichever brought you here, we've made this simple. CARD 1 — FOR PATIENTS & FAMILIES “Recovering from an injury, surgery, or illness? See how OA's coordinated programme works — and how quickly you can start.” → links to Services / For Patients CARD 2 — FOR REFERRING PROFESSIONALS “Looking for an outpatient partner you can trust with continuity of care? See how OA works alongside your practice.” → links to Services / For Practitioners WHAT YOU GET WHEN YOU REFER TO OA — Regular progress & outcome reports — structured updates on your patient's progress against the goals set at discharge. — A seat at our MDT case discussions — an open invitation to join the conversation on shared patients, so your clinical input stays part of the plan. — A direct line to the treating therapist — no call centre, no run-around; you speak directly to the OA clinician managing the case. — A discharge summary back to you — a full summary at the end of the programme, closing the loop on the referral.
Why OA (3 pillars)	— Coordinated, not fragmented — five disciplines under one programme, one point of contact, one recovery plan built around you.

	— Medical aid handled for you — OA is accepted by most major medical aids, and the team manages authorisation so recovery doesn't cost more than it should. — We come to you — included patient transport removes one of the biggest barriers to consistent recovery.
The five disciplines	Short intro: “Whatever your recovery needs, it's under one roof.” Order and imagery requirement: the five disciplines must always appear in the exact sequence used on the Z-fold brochure — Physiotherapy, Occupational Therapy, Clinical Psychology, Speech Therapy, Dietetics — never reordered or alphabetised. Each discipline card must carry a real photograph of an OA practitioner actively working with a patient in that discipline (not a generic icon or stock image standing in for the photo) — this is a hard requirement, not a placeholder option. — Physiotherapy — restoring movement, strength, and physical function after injury, surgery, or stroke. [Image: physiotherapist and patient] — Occupational Therapy — getting you back to the daily tasks that matter: home, work, and independence. [Image: occupational therapist and patient] — Clinical Psychology — evidence-based mental health support, because recovery is never only physical. [Image: clinical psychologist and patient] — Speech Therapy — rebuilding communication and swallowing function after stroke, TBI, or neurological illness. [Image: speech therapist and patient] — Dietetics — personalised, practical nutrition for recovery and chronic condition management. [Image: dietitian and patient]
You don't have to recover alone	Section built around group-therapy photography (per Theo's direction: lead with images, not copy). <i>You don't have to recover alone.</i> OA runs structured group sessions for patients with shared conditions — peer support, shared accountability, and professional guidance, with light meals and refreshments on site.
Resources teaser	“New to recovery, or new to referring? Start here.” — 3-card teaser pulling the most useful Resources entries for each audience, linking through to the Resources page.
Closing CTA band	Full-width dark-teal band, tagline treatment: <i>Supporting Recovery. Promoting Independence. Enhancing Wellbeing.</i> — WhatsApp Us · Call 081 358 2781 · Cnr 8 Morris Avenue and Jolin Avenue, Morehill, Benoni, 1501

6.2 About Us

Hero	HEADLINE Care shouldn't stop at the hospital door. SUBHEAD OA Outpatient Therapy Services was built to close the gap between being discharged from hospital and living independently again — with one coordinated team, instead of five separate appointments to chase.
Our story / why we exist	Every year, patients leave hospital with a discharge letter and very little else — no single person coordinating their physiotherapy, their psychology support, their nutrition, their speech therapy. They're left to find, book, and manage each specialist alone, often while still recovering. OA exists to remove that burden: one programme, one team, coordinated entirely around the patient.
Our approach	— Integrated by design — our five disciplines work as one team around each patient, not as separate practices sharing a waiting room. — Built around the patient, not the diagnosis — programmes are tailored to each person's specific recovery goals, not a generic plan. — Accessible by design — medical aid authorisation handled on the patient's behalf, and included transport so distance and mobility are never the reason recovery stalls.
Where we are	Based in Benoni, Ekurhuleni, serving the greater Johannesburg area. Corner 8 Morris Avenue and Jolin Avenue, Morehill, Benoni, 1501 (use the gate at Jolin Avenue).
Our team	Placeholder section for Phase 1 build — structure only, content to follow from Theo/OA: practitioner photos, names, disciplines, and registration/qualification detail (e.g. HPCSA registration) once supplied. Recommend a simple grid of team cards, one per discipline lead, consistent with the five-discipline icon set from the brochure.
Mission statement (closing)	<i>Supporting Recovery. Promoting Independence. Enhancing Wellbeing.</i>

6.3 Services — For Patients

Hero (problem-led)	HEADLINE You've left hospital. Now what? BODY No single specialist can carry a full recovery on their own — and chasing five different bookings, five different waiting rooms, and five different bills is exhausting when you're already recovering. OA brings every discipline your recovery needs into one coordinated programme, with no referral required to start. CTA WhatsApp Us to Start Your Programme / Call 081 358 2781
The problem, stated plainly	— You don't know which specialist to see first, or whether you need more than one. — Getting to appointments is hard when mobility, energy, or transport is limited. — Medical aid authorisation is confusing, and cost uncertainty adds stress to an already stressful time. — Recovering alone, without peer support, is isolating — especially after a major diagnosis or event.
The OA solution	One integrated programme, five disciplines, coordinated around your specific needs. As on the brochure, the disciplines are presented in this fixed order — Physiotherapy, Occupational Therapy, Clinical Psychology, Speech Therapy, Dietetics — and each one is paired with a real photograph of an OA practitioner working with a patient in that discipline, so visitors see the care itself rather than a generic icon. — Physiotherapy — restoring movement, strength, and physical function after injury, surgery, or stroke. [Image: physiotherapist and patient] — Occupational Therapy — getting you back to the daily tasks that matter: home, work, and independence. [Image: occupational therapist and patient] — Clinical Psychology — evidence-based mental health support for recovery that isn't only physical. [Image: clinical psychologist and patient] — Speech Therapy — rebuilding communication and swallowing function after stroke, TBI, or neurological illness. [Image: speech therapist and patient] — Dietetics — personalised nutrition for recovery and chronic condition management, practical and affordable. [Image: dietitian and patient]
How it works (5 steps)	— 1. Call or WhatsApp us — tell us who you are and what you need. We respond within 24 hours. — 2. We assess your needs — a programme tailored to your condition and recovery goals, never a generic plan. — 3. Your medical aid is sorted — our team handles authorisation, so you can focus on getting better.

	— 4. Transport arranged — we come to you, so you never need to arrange a lift. — 5. Programme begins — you're not alone in this anymore.
Group therapy	<i>You don't have to recover alone.</i> OA runs structured group sessions for patients with shared conditions — peer support, shared accountability, and professional guidance. Light meals and refreshments are available on site, along with a comfortable space to rest between sessions.
Medical aid & payment	MEDICAL AID AUTHORISATION Your medical aid almost certainly covers this. OA is accepted by most major medical aids, and our team manages the authorisation process on your behalf. PRIVATE / CASH PAY Card payments accepted. We also assist with medical aid applications if you're not yet covered.
Transport	Patient transport is included. We collect you from home and bring you back after every session — safe, reliable, and covered by most medical aid transport benefits.
Closing CTA	“Your first step is a phone call.” WhatsApp Us Today / Call 081 358 2781 / Cnr 8 Morris Avenue and Jolin Avenue, Morehill, Benoni, 1501.

6.4 Services — For Practitioners

Hero (problem-led)	HEADLINE Continuing the care you already started. BODY When a patient leaves your care, you lose visibility of what happens next — whether the recovery plan you envisioned actually continues, and whether you'll ever hear how it went. OA proposes a different model: a genuine MDT referral partnership across the full continuum, from acute, to rehab, to outpatient. SUPPORTING LINE (CONFIRMED ALTERNATE, USABLE AS A SUB-HEADLINE OR PULL-QUOTE) “Continuity of care, guaranteed. When you refer a patient to OA, you're not handing them off — you're extending the care you've already started.” CTA Add OA to Your Referral Pathway — WhatsApp Us
The problem, stated plainly	— Referring a patient into outpatient care often means referring into a black box — no update, no report, no way to know if the plan you set is being followed. — Without a coordinated MDT on the receiving end, patients fall through gaps between disciplines instead of moving through a continuum. — There's rarely a direct line back to the treating clinician if something changes in the patient's condition or needs.
The OA solution: a proposed MDT referral loop	OA positions itself as an extension of your own care team within the Acute → Rehab → Outpatient continuum — not a disconnected downstream provider, and not a restart. Your patient's discharge notes and goals feed directly into the OA case plan: the MDT builds on your clinical work, not around it. In return for a referral, you receive four concrete things (per the OA referral partner presentation): — Regular progress & outcome reports — structured updates against the goals set at discharge. — A seat at our MDT case discussions — an open invitation to join the conversation on shared patients, so your clinical input stays part of the plan. — A direct line to the treating therapist — no call centre, no run-around; you speak directly to the OA clinician managing the case. — A discharge summary back to you — a full summary at the end of the programme, closing the loop on the referral.
What OA brings to the continuum	Four labelled benefits, verbatim from the approved referral presentation — use these as the four benefit cards/icons for this section: — A trusted extension of your practice — one coordinated team picks up where your care leaves off; no gaps, no lost momentum. — Every discipline, one programme — Physio, OT, psychology, speech, and dietetics working together, not in isolation.

	— No barrier to attending — transport is arranged for every session, so distance and mobility never break the plan. — Zero admin for you — OA manages medical aid authorisation directly, so referring takes minutes, not hours. — Additional detail to weave in: structured group programmes (stroke, cancer, cardio, psych) add peer accountability alongside each patient's individual plan, and private/cash pay is accepted on-site with help applying for cover where a patient isn't yet covered.
How referring works (5 steps)	A practitioner-specific version of the “How to Get Started” steps — distinct from the patient-facing steps on the Services / For Patients page, and confirmed in the referral partner presentation: — 1. You call or WhatsApp — tell us who the patient is and what they need. We respond within 24 hours. — 2. We build on your plan — the programme is tailored around the discharge notes and goals you provide, not a restart. — 3. Medical aid is sorted — our team handles authorisation directly, so the patient starts without delay. — 4. Transport arranged — we come to them; no missed sessions due to access. — 5. MDT loop opens — you get visibility from day one: reports, case discussions, and a direct line to the therapist.
Who this is for	— Hospital social workers and discharge planners (public and private hospitals). — General practitioners and specialists needing a trusted outpatient rehab and allied-health partner. — Oncology units and cancer care teams, particularly for psychology referrals that are too often skipped. — Physiotherapy and rehabilitation centres seeking a multidisciplinary step-down partner.
Call to action	“Add OA to your formal referral pathway.” Primary CTA: WhatsApp Us to discuss a referral partnership. Secondary: Download the OA Referral Partner one-pager (PDF, drawn from the approved referral presentation) / Call 081 358 2781.

6.5 Resources

Hero	HEADLINE Straightforward answers, for patients and professionals. BODY A growing library of practical guides and answers — filter by whether you're a patient/family member or a referring professional.
Content pillars — for patients	— What to expect at your first OA assessment. — Recovering at home after a stroke, surgery, or major illness — practical guides. — Medical aid & OA: authorisation, cover, and what it costs if you're not covered. — What group therapy at OA actually looks like. — Frequently asked questions (transport, timing, what to bring).
Content pillars — for referring professionals	— The OA referral pathway, explained (acute → rehab → outpatient continuum). — What you receive when you refer to OA: progress reports, MDT case discussions, discharge summaries. — Downloadable referral partner one-pager (PDF). — Cancer & mental health: why psychology referrals matter post-diagnosis (ties to OA's community awareness campaign work).
Structure & UX note	Simple filterable/tabbed layout: “For Patients” / “For Professionals” toggle at the top of the page. Each resource is a short, scannable article — 300–500 words, image-led where possible, ending in the relevant WhatsApp CTA for that audience. This section also does double duty as the site's primary SEO surface (see Section 8).

6.6 Contact Us

Hero	HEADLINE Your first step is a phone call — or a WhatsApp message. BODY Tell us who you are and what you need. We respond within 24 hours.
Contact details	— Phone / WhatsApp: 081 358 2781 — Email: info@oatherapy.co.za — Address: Corner 8 Morris Avenue and Jolin Avenue, Morehill, Benoni, 1501 (use the gate at Jolin Avenue) — Social: @oatherapy (Facebook, Instagram, LinkedIn)
Contact form	A single form with an audience selector, so enquiries route correctly from the start: — “I am a...” — Patient / Family Member · Referring Doctor or Hospital · Allied Health Professional · Other — Name, phone, email, message — Conditional field for professionals: practice/hospital name — Submit button: “Send to OA” — with a confirmation state that also surfaces the WhatsApp option (“Prefer to chat now? WhatsApp us instead.”)
Map & directions	Embedded map pinned to Corner 8 Morris Avenue and Jolin Avenue, Morehill, Benoni, 1501, with a visible note: “Use the gate at Jolin Avenue.”

7. WhatsApp Integration Specification

WhatsApp is OA's primary patient and referrer communication channel (confirmed in the OA business model document) and the site's number-one conversion goal. Phase 1 uses simple, reliable click-to-chat links — no chatbot/automation build required yet (see Phase 2).

7.1 Site-wide placement

- Persistent header button, all pages, all breakpoints: “WhatsApp Us.”
- Fixed bottom bar on mobile: “WhatsApp” + “Call” side by side, always visible while scrolling.
- A floating action button (bottom-right, standard WhatsApp green-accented within the OA palette) on desktop as a secondary, non-intrusive prompt.

7.2 Mechanism (Phase 1)

Standard WhatsApp “click-to-chat” links (wa.me/27813582781) — no API integration required. Each placement should carry a pre-filled, context-specific message so the visitor doesn't have to type a cold open:

Placement	Pre-filled message
Homepage / general	“Hi, I'd like to know more about OA's programme.”
Services — For Patients	“Hi, I'd like to start therapy at OA — can you tell me what's involved?”
Services — For Practitioners	“Hi, I'm a healthcare professional and I'd like to refer a patient to OA / discuss a referral partnership.”
Resources	“Hi, I had a question after reading one of your resource articles.”
Contact Us	“Hi, I'd like to get in touch with OA.”

8. SEO & Content Notes

Target phrases to weave naturally into page copy, headings, and Resources articles — never keyword-stuffed:

- Outpatient therapy Benoni / Ekurhuleni
- Physiotherapy after stroke Johannesburg
- Occupational therapy after hospital discharge
- Speech therapy after stroke Benoni
- Allied health referral pathway Johannesburg
- Group therapy for cancer patients Gauteng

Each page should carry a unique meta title and description reflecting the audience it's written for (patient-facing pages should mention location and condition keywords; the Practitioners page should mention “referral pathway” and “MDT”).

9. Phase 2 — Noted for Later (Not in This Build)

- Online booking / appointment scheduling.
- Patient or referrer portal (progress reports, discharge summaries delivered digitally).
- WhatsApp Business API automation (auto-replies, intake triage, appointment reminders).
- Team page with full practitioner bios, photos, and registration numbers, once supplied.
- Testimonials / case studies (patient consent required).
- Downloadable referral partner PDF as a gated resource with basic lead capture.

10. Appendix — Assets Needed From OA / Theo

- Logo files (vector — SVG/AI/EPS) and full brand guide, if one exists beyond the brochure/deck.
- Real photography: group sessions, individual therapy, the facility, the branded transport vehicle, the team — per Theo's "images over copy" direction.
- One photograph per discipline (five total) showing an OA practitioner actively working with a patient — Physiotherapy, Occupational Therapy, Clinical Psychology, Speech Therapy, Dietetics, in that order — for the discipline cards on Home and Services / For Patients.
- Team member names, photos, disciplines, and qualifications/registration numbers for the About Us and (future) team page.
- Final, confirmed list of medical aids accepted (if OA wants to name specific schemes rather than "most major medical aids").
- Any existing patient testimonials or consented case studies.
- Confirmation of WhatsApp number to use site-wide (assumed 081 358 2781, per brochure).
- Sign-off on Cambria/Calibri as web fonts vs. approved web-safe substitutes (Section 3.2).